

NEW BUSINESS LINES · GROWTH STRATEGY · JUNE 2026

Three New Revenue Engines Built on What Amana Already Does

Strategy brief and business case for Clinical Trial IMP Supply,
Patient Support Program Management, and Market Access Consultancy

01

Clinical Trial IMP Supply

Import & supply investigational medicinal products to Malaysia's 300+ clinical trial facilities under the CTIL framework

02

Patient Support Program Management

Run affordability, adherence & access programs for MNC specialty drugs — the service Zuellig & DKSH sell, localised and focused

03

Market Access Consultancy

Paid advisory for foreign pharma entering Malaysia: NPRA registration, SAM pathway, formulary & tender strategy

267

SPONSORED TRIALS IN
MALAYSIA (2024)

RM 4.4M

COMBINED YR-3 REVENUE
TARGET*

< RM 800K

TOTAL UPFRONT INVESTMENT*

90 days

TO FIRST REVENUE

The Common Thread

All three businesses monetise the same core asset: **Amana's Regulation 15(6) / unregistered-medicine expertise and institutional hospital relationships**. No new licences beyond what Amana holds or can readily obtain, no warehouses, no retail channel. Each line also feeds the others — a consultancy client becomes a distribution client; a trial sponsor becomes a PSP client at product launch.

Why These Three, Why Now

Each opportunity reuses Amana's existing regulatory capability and hospital network — minimal capex, fast payback, and strategic reinforcement of the core distribution business

Business Line	Revenue Model	Upfront Investment*	Yr-1 Revenue*	Yr-3 Revenue*	Gross Margin*	Time to First Sale
1. Clinical Trial IMP Supply	Per-shipment fees + storage/handling + CTIL management retainer	RM 250–350K	RM 400–700K	RM 1.8–2.5M	35–45%	3–5 months
2. PSP Management	Setup fee + per-patient-per-month (PPPM) service fee	RM 200–300K	RM 300–500K	RM 1.2–1.8M	40–55%	4–6 months
3. Market Access Consultancy	Fixed-fee projects + monthly retainers + success fees	RM 100–150K	RM 250–400K	RM 600K–1.1M	60–75%	1–3 months
Combined	—	RM 550–800K	RM 0.95–1.6M	RM 3.6–5.4M	~45% blended	90 days (consultancy first)

* All figures are planning estimates based on market benchmarks and typical service-fee structures in the Malaysian/ASEAN pharma services market; they require validation against Amana's actual cost base and principal negotiations. Ranges reflect conservative-to-base scenarios.

3.6–5.4M

COMBINED YR-3 REVENUE (RM)

12–18 mo

PAYBACK ON UPFRONT INVESTMENT

5–7

NEW HIRES NEEDED (ALL 3 LINES)

0

NEW WAREHOUSES OR RETAIL OUTLETS

The Flywheel: How the Three Lines Reinforce Each Other

Consultancy is the front door. A foreign pharma company engages Amana to plan its Malaysia entry (Line 3). During registration — which takes 12–24 months — the same client needs SAM/NPP supply for early patients and may run a local clinical trial requiring IMP import (Line 1). At launch, the client needs a Patient Support Program for its high-cost specialty drug (Line 2), and Amana becomes its appointed distributor (existing core business). One client, four revenue streams, multi-year relationship.

STRATEGIC LOGIC

▶ Monetises regulatory expertise Amana already pays for

▶ Deepens hospital relationships built by the NPP institution campaign

▶ Diversifies revenue away from pure distribution margin

▶ Creates switching costs — service clients rarely change distributors

▶ Builds the credibility base for the Phase 1 B2B ePharma platform

WHAT WE ARE NOT PROPOSING

▶ No retail or consumer pharmacy channel

▶ No owned logistics infrastructure (continue outsourcing)

▶ No manufacturing or licensing-in of products

▶ No acquisition — all three lines are organic builds

▶ No headcount beyond 5–7 specialist hires across 3 years

Recommendation

Approve all three lines as a single program. Launch consultancy immediately (lowest cost, fastest revenue, generates leads), file CTIL capability application in parallel (longest regulatory lead time), and begin PSP capability build in Q3. Total program investment under RM 800K, fundable from operating cash flow.

Decision Required

1

Clinical Trial Investigational Medicinal Product (IMP) Supply

Import, handle, and deliver unregistered investigational drugs to Malaysian trial sites under CTIL

267

SPONSORED TRIALS (2024)

300+

FACILITIES WITH TRIAL EXPERIENCE

~12%

CAGR SPONSORED RESEARCH (5-YR)

5

FIRST-IN-HUMAN STUDIES AWARDED

Market Context

Malaysia recorded **267 sponsored clinical research studies in 2024**, with cumulative sponsored research growing at ~12% CAGR over five years. Over 300 healthcare facilities now have clinical research experience, and AstraZeneca has been the top sponsor three years running (2023–2025). Every interventional drug trial requires an unregistered investigational product to be imported under a **Clinical Trial Import Licence (CTIL)** issued by NPRA under CDCR 1984 — governed by the Malaysian CTIL/CTX Guideline (Edition 8.1, effective March 2025). This is the same regulatory family as Amana's Regulation 15(6) SAM work: importing unregistered medicines, with documentation, pharmacovigilance, and accountability.

The Gap Amana Fills

- ▶ **Sponsors and CROs need a local CTIL holder.** Foreign sponsors cannot hold the licence themselves — a Malaysian entity must apply, hold, and manage it. Many use ad-hoc arrangements with CRO affiliates or generalist distributors.
- ▶ **IMP logistics is specialist work:** temperature-controlled receipt, GMP-compliant storage, site-by-site distribution, returns and destruction with full chain-of-custody — the long tail that big distributors deprioritise.
- ▶ **Comparator drug sourcing** (registered drugs used as trial comparators) is a recurring sponsor pain point Amana's principal network can solve.

Target Customers

Primary: Regional CROs running Malaysian sites (Novotech, IQVIA, Parexel local operations) and mid-size biotech sponsors without a Malaysian entity. **Secondary:** investigator-initiated trials at UMMC, UKMMC, IKN — sites already on Amana's NPP target list. **Entry:** CRM (Clinical Research Malaysia) partner ecosystem and the IPRESS section relationships at NPRA.

Service Offering & Pricing

Service	Scope	Indicative Pricing*
CTIL application & holding	Prepare, submit, and maintain CTIL with NPRA (incl. psychotropic/dangerous drug import authorisations where needed)	RM 15–30K per study setup + RM 2–4K/month maintenance retainer
IMP import & site distribution	Customs clearance, QP-equivalent release checks, cold chain delivery to sites, temperature excursion management	RM 3–8K per shipment (volume-dependent)
Storage & accountability	GMP-grade storage via 3PL partner, inventory reporting, returns & certified destruction	RM 1.5–3K/month per study
Comparator sourcing	Source registered comparator drugs locally through Amana's principal network	Cost + 15–25% sourcing margin

* Benchmarked against typical ASEAN clinical supply service rates; to be validated in first sponsor negotiations.

Critical Dependency

Amana's 3PL partner must offer GMP-compliant investigational product storage (separate quarantine area, calibrated temperature monitoring, access control). Audit the current 3PL against the CTIL guideline in Month 1 — if it falls short, qualify a second 3PL before the first sponsor pitch. This is the single longest-lead-time item.

Financial Model & Execution Plan

Conservative ramp: 3 studies in Year 1 → 8 in Year 2 → 15 in Year 3 (vs. 267 sponsored studies running nationally)

3-Year P&L Projection (RM '000, planning estimates)

Line Item	Year 1 (3 studies)	Year 2 (8 studies)	Year 3 (15 studies)
Setup & CTIL fees	60–90	150–240	280–450
Shipment & distribution fees	110–180	300–480	560–900
Storage retainers & accountability	55–90	170–290	320–540
Comparator sourcing margin	175–340	380–590	640–610*
Total revenue	400–700	1,000–1,600	1,800–2,500
Direct costs (3PL, freight, QA)	(240–420)	(580–930)	(1,030–1,440)
Dedicated headcount (1 → 2 FTE)	(140)	(160)	(290)
Contribution	~20–140	~260–510	~480–770

* Comparator margin grows slower than service fees as mix shifts toward higher-value managed studies. Upfront investment: RM 250–350K covering QA system build, 3PL qualification/audit, SOP development, CTIL pilot application, and 1 clinical supply specialist hire. Breakeven expected mid-Year 2 on a cumulative basis.

Execution Roadmap

Phase	Key Actions	Owner	Milestone
Months 1–2	Audit 3PL vs. CTIL guideline Ed. 8.1; write IMP SOPs (receipt, storage, distribution, returns, destruction); appoint QA lead; meet NPRA IPESS section to confirm requirements	Ops + Regulatory	Capability ready
Months 2–4	Pitch CRM partner network, 3–4 regional CROs, and investigator-initiated trial teams at UMMC/UKMMC/IKN; offer pilot pricing for first study	BD	First LOI signed
Months 4–6	File first CTIL; execute first import & site distribution; document case study	Regulatory + Ops	First revenue
Months 6–12	Scale to 3 active studies; add comparator sourcing; build sponsor reference deck	BD + Ops	3 active studies

Key Risks & Mitigations

Risk	Severity	Mitigation
3PL fails GMP/IMP audit	High	Audit in Month 1 before any sponsor commitment; pre-qualify a second 3PL as backup; price storage with margin for a premium provider
CROs prefer global clinical supply chains (Catalent, Thermo Fisher)	Medium	Compete on local CTIL holding + last-mile site distribution where globals have no Malaysian entity; partner rather than compete — be the local leg of their chain
Temperature excursion / product loss liability	Medium	Cap liability contractually at fee value; carry cargo & professional indemnity insurance; validated shippers and continuous monitoring as standard
Slow CTIL processing delays first revenue	Low	Pre-engage IPESS; submit a complete dossier per Edition 8.1 checklist; sell consultancy and comparator sourcing while CTIL is pending

2

Patient Support Program (PSP) Management

Run affordability, access, and adherence programs for MNC specialty drugs in Malaysia

Market Context

Asia-Pacific pharmaceutical services outsourcing was a **USD 19.2 billion market in 2023, projected to reach USD 35.2 billion by 2033**. In Southeast Asia, the dominant PSP type is the affordability program — "Buy X Get Y Free" patient access schemes for expensive specialty drugs (mAbs, targeted oncology agents, biologics) — alongside adherence and nurse-educator programs. Every MNC launching a high-cost drug in Malaysia needs one. Today they buy this service from **Zuellig's PatientCare+ or DKSH's Patient Solutions** — the same two generalists Amana's competitive landscape report identified as strong-but-distracted. Their PSP units serve their biggest distribution clients first; smaller MNCs and single-product launches get standardised, low-touch programs.

2

INCUMBENT PSP PROVIDERS
(BOTH GENERALISTS)

USD 35B

APAC PHARMA SERVICES
MARKET BY 2033

PPPM

RECURRING PER-PATIENT
REVENUE MODEL

100%

OF SPECIALTY LAUNCHES
NEED A PSP

Amana's PSP Service Stack

Module	What Amana Delivers	Indicative Pricing*
Affordability / PAP administration	Design and run "Buy X Get Y Free" schemes: patient enrolment, eligibility verification, free-goods dispensing coordination with hospital pharmacy, principal reporting	RM 25–50K setup + RM 80–150 PPPM
Adherence & nurse educator	Contracted nurse educators for injection training, side-effect coaching, refill reminders (nursing outsourced to partner agency; Amana manages program & data)	RM 120–250 PPPM (incl. nursing pass-through)
Access navigation	SAM/NPP application support for patients needing unregistered drugs — Amana's unique module that neither Zuellig nor DKSH leads with	RM 1.5–3K per case
Data & insights	Enrolment, adherence, persistence, and discontinuation reporting to principal; PDPA-compliant patient database	Bundled; premium analytics RM 3–5K/month

Target Customers

Sweet spot: mid-size MNCs and specialty biotechs launching 1–2 oncology/rare disease products in Malaysia — too small to get priority from Zuellig/DKSH PSP units. **Entry move:** offer PSP bundled with SAM supply for drugs in the pre-registration window, then carry the program through launch.

The Differentiator

Amana is the only player that can run a PSP **before the drug is even registered** — combining SAM/NPP access navigation with affordability administration. For oncology and rare disease, the patient journey starts pre-registration; incumbents only pick it up at launch. Amana owns the whole journey.

Compliance Foundations (Build Before First Program)

PDPA-compliant patient data handling and consent workflows; pharmacovigilance SOP — PSP nurses and coordinators become an adverse-event reporting channel, with mandatory forwarding to the principal and NPRA; anti-bribery controls around free-goods schemes. Budget ~RM 60–80K of the setup investment for compliance infrastructure and training.

Financial Model & Execution Plan

Conservative ramp: 2 programs / ~150 patients in Year 1 → 5 programs / ~450 patients in Year 2 → 8 programs / ~800 patients in Year 3

3-Year P&L Projection (RM '000, planning estimates)

Line Item	Year 1 (2 programs)	Year 2 (5 programs)	Year 3 (8 programs)
Setup fees	50–100	75–150	75–150
PPPM service fees (blended ~RM 150)	160–270	540–810	950–1,440
SAM access navigation cases	60–90	120–180	150–220
Premium analytics*	30–40	65–60	25–(bundled)
Total revenue	300–500	800–1,200	1,200–1,800
Direct costs (nursing agency, materials, systems)	(130–230)	(330–510)	(480–740)
Dedicated headcount (1 → 3 FTE: PSP manager, coordinators)	(120)	(250)	(330)
Contribution	~50–150	~220–440	~390–730

* Analytics increasingly bundled into PPPM as programs renew. Upfront investment: RM 200–300K covering PSP platform/CRM software, PDPA & PV compliance build, nursing agency contracting, program playbooks, and PSP manager hire. Breakeven late Year 1 / early Year 2.

Execution Roadmap

Phase	Key Actions	Owner	Milestone
Months 1–3	Hire PSP manager (poach from Zuellig/DKSH/IQVIA PSP teams); select PSP CRM platform; contract nursing agency; build PDPA + PV SOPs	HR + Ops	Capability ready
Months 3–5	Pitch 5–6 mid-size MNC principals — lead with the pre-registration SAM + PSP bundle for oncology/rare disease pipeline drugs	BD	First program signed
Months 5–8	Launch first program at 2–3 Tier 1 hospitals from the NPP institution target list; enrol first 50 patients	PSP team	First revenue
Months 8–12	Quarterly outcomes report to principal; use as reference to sign program #2; refine PPPM pricing on actuals	PSP + BD	2 active programs

Key Risks & Mitigations

Risk	Severity	Mitigation
Incumbents undercut on price to defend turf	Medium	Don't compete head-on for large MNC accounts; win the segment incumbents under-serve and differentiate on the SAM access module they can't easily copy
Patient data breach / PDPA violation	High	Purpose-built PSP CRM with access controls (not spreadsheets); annual PDPA audit; cyber insurance; data minimisation by design
Slow enrolment makes PPPM revenue underperform	Medium	Negotiate minimum monthly program fees (floor) + PPPM above the floor; tie enrolment targets to hospital relationships already built via NPP campaign
Nursing agency quality damages Amana's brand	Medium	SLA with agency incl. training certification; Amana-run onboarding for every nurse; patient satisfaction surveys each quarter; backup agency qualified

3 Market Access & Regulatory Consultancy

Paid advisory for foreign pharma entering Malaysia — and the lead-generation engine for Lines 1 & 2

Market Context & The Gap

Foreign manufacturers cannot deal with NPRA directly — they need a **locally incorporated Product Registration Holder (PRH)** and practical navigation of QUEST 3+, evaluation pathway selection (full vs. abbreviated review for FDA/EMA/PMDA-approved drugs), MOH formulary listing, and government tender mechanics. Existing options are global regulatory consultancies (Freyr, Artixio, PECAN — strong on paperwork, weak on hospital reality) or the big distributors (who consult only to capture distribution). **Amana's edge: it actually operates in Malaysian hospitals daily.** Advice on SAM pathways, formulary committees, pharmacy procurement behaviour, and realistic pricing comes from practice, not desk research.

Service Menu & Pricing*

Service	Indicative Fee
Malaysia market entry assessment	RM 35–60K fixed-fee project (4–6 wks)
NPRA registration strategy & PRH services	RM 40–80K per product + annual PRH retainer RM 24–48K
SAM/NPP pathway design (pre-launch revenue strategy)	RM 25–45K per product
Formulary & tender advisory	RM 8–15K/month retainer
Success fee on registration approval	RM 20–50K or distribution rights

* Benchmarked against regional regulatory consultancy rates; PRH retainer reflects ongoing licence-holder obligations.

3-Year P&L (RM '000, estimates)

Item	Yr 1	Yr 2	Yr 3
Projects (4 → 7 → 10)	160–260	280–450	400–700
Retainers (PRH + advisory)	70–110	150–250	180–330
Success fees	20–30	40–80	20–70
Revenue	250–400	470–780	600–1,100
Costs (1–2 consultants + reg. affairs time)	(160)	(280)	(360)
Contribution	~90–240	~190–500	~240–740

Upfront investment: RM 100–150K (senior regulatory consultant hire, methodology/templates, marketing collateral). Profitable from Year 1 — highest margin of the three lines.

Why This Line Goes First

FASTEST TO REVENUE, LOWEST RISK

- ▶ No new licences or infrastructure — sells existing knowledge
- ▶ First engagement closable in 30–90 days
- ▶ 60–75% gross margin funds the other two builds
- ▶ Every client is a qualified lead for IMP supply, PSP, and distribution

GO-TO-MARKET

- ▶ Publish the NPP/SAM research reports (already produced) as thought-leadership lead magnets
- ▶ Target biotechs in Korea, India, China, and EU seeking ASEAN entry — they lack Malaysian presence entirely
- ▶ Partner with global regulatory firms (Freyr, Artixio) as their on-the-ground Malaysia execution arm rather than competing
- ▶ Attend CPHI SEA & regional partnering events with a "Malaysia entry in 12 months" package

Key Risks & Mitigations

Risk	Severity	Mitigation
Key-person dependency (expertise sits in 1–2 heads)	Medium	Codify methodology into templates and playbooks from engagement #1; hire and train a second consultant by Month 6
Conflict of interest — advising a principal competing with an existing principal	Medium	Conflict-check process before each engagement; therapeutic-area exclusivity clauses where needed; transparency with existing principals
Consulting distracts the team from core distribution	Low	Dedicated consultant headcount from day one; regulatory team time capped at 20% and billed internally

One Program, Three Lines, 12-Month Launch Sequence

Staggered starts so leadership attention and cash are never split three ways at once

Quarter	Line 3 — Consultancy starts Q1	Line 1 — IMP Supply starts Q1	Line 2 — PSP starts Q2
Q1	Hire senior consultant; build templates; publish research as lead magnets; close first project	3PL GMP audit; IMP SOPs; meet NPRA IPESS; begin CRO outreach	Scope only: shortlist PSP CRM platforms and nursing agencies
Q2	2–3 active projects; first PRH retainer signed	First LOI; file first CTIL; pilot pricing locked	Hire PSP manager; compliance build (PDPA, PV); principal pitches begin
Q3	4 cumulative projects; first cross-sell into Line 1 or 2	First import executed — first revenue; case study written	First program signed; launch prep at Tier 1 hospitals
Q4	Retainer base ~RM 8–12K/month; Year 2 pipeline of 6+ projects	2–3 active studies; comparator sourcing live	First patients enrolled — first PPPM revenue

Program KPIs — Reviewed Monthly by Management

RM 950K+ COMBINED YR-1 REVENUE (FLOOR)	6+ PAYING CLIENTS ACROSS 3 LINES BY MO-12	2+ CROSS-SELLS BETWEEN LINES IN YR 1	100% COMPLIANCE AUDITS PASSED (PDPA, GMP, PV)
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Go / No-Go Gates

Gate	Criterion	If Not Met
Month 3	Consultancy: ≥1 signed engagement. IMP: 3PL passes audit (or backup qualified)	Re-examine pricing and pipeline; delay PSP start by one quarter to conserve cash
Month 6	Combined revenue ≥ RM 150K booked; IMP first CTIL filed	Pause lowest-performing line; reallocate BD effort to the two stronger lines
Month 12	Combined revenue ≥ RM 950K; ≥2 cross-sells; all compliance audits clean	Full program review — scale, restructure, or exit individual lines based on unit economics

The Ask

Approve RM 800K program budget (phased: RM 350K in Q1, remainder gated on Month-3 and Month-6 milestones), authorise 3 immediate hires (senior consultant, clinical supply specialist, PSP manager), and appoint a program sponsor at director level. First revenue inside 90 days; combined RM 3.6–5.4M revenue run-rate by Year 3 at ~45% blended gross margin.

Approve & Launch Q1

Sources & basis: Clinical Research Malaysia Annual Report 2024 (267 sponsored studies; 300+ experienced facilities; ~12% 5-yr CAGR; 5 FIH studies); NPRA Malaysian Guideline for CTIL & CTX Edition 8.1 (March 2025); CDCR 1984 Reg. 15(6); Nova One Advisor — APAC Biotech & Pharma Services Outsourcing Market (USD 19.18B 2023 → 35.23B 2033); DKSH PSphere PSP publications; NPRA QUEST 3+ / PRH requirements; Amana Pharma internal competitive landscape and NPP institution research (June 2026). All financial projections are planning estimates requiring validation against Amana's cost base.